

Our Ref: (T)BGM /BP/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

INTERSTITIAL LUNG DISEASE

[PROVIDER-NAME] and Honorary Senior Clinical Lecturer

[PROVIDER-NAME] and Honorary Associate Professor in Medicine

[PROVIDER-NAME]

[PROVIDER-NAME]

Division B

Care Group - Emergency & Specialist Medicine Respiratory Medicine

Minerva House, 2nd Floor, Mailpoint

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

023 8120 xxxx -->

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Pulmonary sarcoidosis

Inflammatory bowel disease currently quiescent (jejunal resection 2015)

Current lower respiratory tract infection

[NAME] came back to the clinic today. For the last three weeks he has had an increased cough symptoms and he has difficulty sleeping. His wife and daughter were also affected after a holiday in Turkey.

A chest x-ray shows patchy change in the right lower zone consistent with infection. As he is on Azathioprine 25mgs once a day and low dose Prednisolone I have given him a week of Amoxicillin to increase his Prednisolone to 30mgs once a day. He has polyuria but there is no evidence of glycosuria on the dipstick of his urine today.

His calcium has not been elevated in the past either. I have asked him to have a repeat chest x-ray at the end of October and I will look at it at that stage. Otherwise I will see him in the new year.

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Respiratory Physician and

Honorary Associate Professor in Medicine